



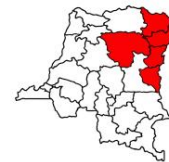
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°084/MVEBDB/06/08/2026

Reporting date: August 6, 2026

Publication date: August 7, 2026



HIGHLIGHTS (last 24 hours)

Over the past 24 hours, 67 new confirmed cases of Ebola Virus Disease (EVD) have been recorded in the provinces of Ituri (42 cases), North Kivu (11 cases), Haut-Uélé (13 cases) and Tshopo (1 case), reflecting the continuation of active transmission, mainly in Ituri.

CUMUL CASES 4,120	ACCUMULATIVE DEATH CONFIRMED** 1887	LETHALITY 45.8%	PATIENTS IN ISOLATION / CTE 721	ACCUMULATIVE HEALED 810	TRACKING RATE CONTACTS (Of the Day) 83.7%
-----------------------------	---	---------------------------	---	-----------------------------------	--



Affected Provinces and Affected Health Zones

5 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu & Tshopo



53 Health Zones
affected Out of 140
(37.8%)



246 Health Areas
touched
Out of 3104 (30%)

- In the past 24 hours, 67 new confirmed cases (15 new deaths) of Ebola Virus Disease (EVD) have been recorded in the provinces of Ituri (42 cases), North Kivu (11 cases) and Haut-Uélé (13 cases) and Tshopo (1) reflecting the continuation of active transmission, mainly in Ituri.
- To date, the Democratic Republic of Congo has recorded 4,120 confirmed cases, including 1,887 deaths, representing a case fatality rate of 45.8%. Ituri remains the epicenter of the epidemic, accounting for 86.5% of cumulative confirmed cases and 62.7% of new cases reported in the last 24 hours.
- The epidemic has now affected 53 of the 140 health zones in the five affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (6/23) and South Kivu (1/34), confirming the continued geographical spread of the disease.
- The National Coordination continues to strengthen surveillance, care, prevention and infection control interventions as well as community engagement in the most affected health zones in order to rapidly interrupt transmission chains.

Distribution of confirmed cases and deaths by affected province

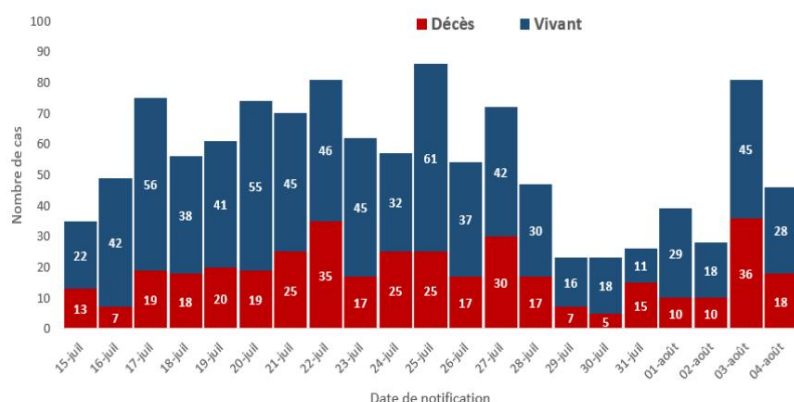
(as of August 6, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	3,563	1,526	42.8% 28/36	(77.8%) 67.9% 12/34	42
North Kivu	458	311	(35.3%) 50.6%	6/13 (46.1%)	11
Haut-Uélé	87	44	55.6% 6/23	(26.0%) 33.3% 1/34	13
Tshopo	9	5	(2.9%)		1
South Kivu	3	1			0
Total	4,120	1887	45.8%	53/140 (37.8%)	67

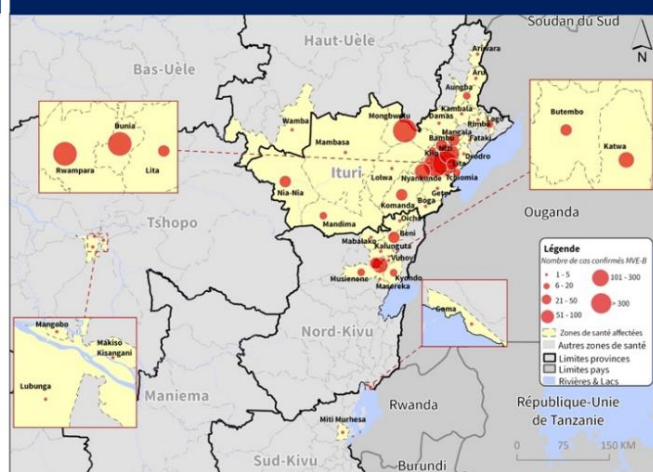
Status of alerts notified by ZS, as of August 6, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	22	3	25	10	3	0	0	13	5
Ituri	462	48	510	282	47	0	0	181	121
North Kivu	559	26	585	559	26	116	4	66	37
South Kivu	2	0	2	2	0	2	0	2	0
Tshopo	19	0	19	19	0	0	0	2	2
Total General	1064	77	1,141	872	76	118	4	264	165

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- Ituri: Validation of data between the Surveillance, Laboratory, and Planning pillars and continued collection of data from health zones; strategic actions remain pending, notably the establishment of sub-coordination units in Wamba, Pawa, and Watsa.
- North Kivu: Coordination in Musienene, harmonization of alert and contact databases, and training of surveillance focal points.
- Haut-Uélé: Coordination meetings held, reporting from 4 out of 5 health zones (80%), and follow-up on ongoing recommendations.

1.1.2. Challenges

Delays in operationalizing the zonal approach and insufficient completeness of reporting, limiting local coordination, comprehensive data collection and rapid decision-making in the health zones concerned.

1.2. Epidemiological surveillance

1.2.1. Main Actions

- 1,141 alerts were recorded, of which 1,070 were verified within 24 hours (93.8%), approaching the 95% target. Of these alerts, 948 were validated as suspected cases.

- Contact tracing is at 83.7%, approaching the 85% threshold, with particularly low performance in Haut-Uélé (50.1%), Nord-Kivu (79.4%) and Ituri (88.9%).

1.2.2. Challenges

Poor performance of epidemiological surveillance, characterized by an alert investigation rate (27.8%) compromising early detection of cases and interruption of transmission chains.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

Two alerts were notified to the PoE/PoC (one in Ituri, one in North Kivu). In Ituri, one live alert was validated and referred to care facilities, while the alert in North Kivu was invalidated. No reports were received in Haut-Uélé or Tshopo. 108,736 travelers were screened, with a screening rate of 95.6%, which is satisfactory in the provinces reporting data.

The operationalization of PoE/PoC remains insufficient in Haut-Uélé (9/38; 23.7%) and Tshopo (6/19; 31.6%), unlike Ituri, North and South Kivu where all priority points are functional.

1.3.2. Challenges

Insufficient PoE/PoC coverage in Haut-Uélé and Tshopo, combined with low sensitivity of the alert detection system in some provinces, limits the effectiveness of surveillance at borders and along mobility routes.

1.4. Laboratory

1.4.1. Main actions

- **360 samples** were analyzed, confirming **67 new cases** of Ebola virus disease (EVD). • Ituri recorded **42 positive results out of 202 samples** (positivity rate: **20.8 %**), while **North Kivu** and **Haut-Uélé** confirmed **11** and **13 cases respectively**. • **Tshopo** reported only one confirmed positive case but continued strengthening diagnostic capacity and receiving samples from Bas-Uélé.

1.4.2. Challenges

High proportion of positive samples in Ituri and incomplete reporting of laboratory results in some provinces, limiting biological surveillance and real-time monitoring of Ebola transmission.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In **Ituri**, 27 rings and 43 sites were decontaminated, 44 health facilities received infection prevention and control (IPC) support, and 112 providers were trained, despite low reporting completeness (97.7%). • In **North Kivu**, 95 health facilities received support and 378 providers were briefed; 46 blood tests were conducted, and 43 bodies were swabbed and secured. • In **Tshopo**, only one facility reached the 80% threshold (Emergency Treatment Center, 82%), and 31 remain below 50% based on assessments conducted in the last 24 hours.

1.5.2. Challenges

Weak implementation of Infection Prevention and Control (IPC) standards, characterized by low reporting completeness in Ituri and insufficient performance of health facilities in Tshopo, limiting the effectiveness of measures to prevent healthcare-associated infections.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 29 ambulances were assigned to the response, of which 24 were available. A total of 45 missions were successfully completed, allowing for the evacuation of 45 patients. The occupancy rate of healthcare facilities reached 488 out of 833 beds (59%).
- **In North Kivu**, a total of 235 admissions were recorded in the CTE/CTs, including 187 confirmed cases, with an intra-CTE mortality rate of 39%.

1.6.2. Challenges

Insufficient availability of patient referral resources and poor completeness of reporting of continuity of care activities, limiting rapid access to care and monitoring of pillar performance.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 4,578 people sensitized, 162 community alerts received, 25 feedbacks out of 65 (38%) clarified and updated, 47 RECO.
- **In North Kivu**, 220 leaders of women's associations and 26 women leaders were briefed, 16 rumors clarified and 1 community incident resolved.
- **In Tshopo**, 900 RECOs were trained in the 5 urban health zones and 423 people reached during home visits.

1.7.2. Challenges

Persistent community challenges, including tensions related to payment of service providers and local resistance, may affect community buy-in and the effective implementation of response interventions.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed treatment capacity has reached 833 beds, including 411 for confirmed cases; the Nia Nia Ebola Treatment Center (ETC) is nearing completion, and requests have been made to expand the Bambu, Nizi, and Logo ETCs. Deliveries of infection prevention and control (IPC/WASH) kits and treatment supplies have been made to the Rwampara and Mongbwalu ETCs, and two mini Starlink kits have been deployed in the Nyankunde health zone.

In North Kivu, the unpacking of the WHO's donation of IPC kits from Bunia continued, and the order for the Kyondo health zone was received. Construction work on the tank bases is ongoing at the Karuruma and Komba checkpoints.

In Tshopo, logistical capacities were strengthened by the delivery of an initial batch of 10 motorcycles and the official handover of an ambulance and motorcycles by the Provincial Governor, as well as by the receipt of 20 thermoflash devices and 10 mattresses.

1.8.2. Challenges

Persistent logistical needs to support the expansion of the response, including the completion of support infrastructure, the maintenance of essential input supplies and the continued availability of means of transport.

1.9. Security 1.9.1.

Main actions

- Community resistance continues to disrupt the response, with failures to provide dignified and safe burials in Bambu, Nyankunde, Rwampara and Tchomia in Ituri, and in Kalunguta in North Kivu.
- In North Kivu, 7 security escorts were provided to the burial, communication and IPC teams; the presence of armed groups still limits humanitarian access.

1.9.2. The challenges

Persistent insecurity and community resistance during safe and dignified burials, limiting access for response teams, continuity of interventions and coverage of activities in some affected areas.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continuation of **briefing and awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA)** in the different provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse remains insufficient across all areas covered by the response.

KEY MESSAGES

- Transmission of Ebola virus disease remains active, with 67 new confirmed cases and 42 deaths recorded in the last 24 hours, bringing the national total to 4120 confirmed cases and 1887 deaths.
- Ituri remains the epicenter of the epidemic, accounting for 86.5% of cumulative confirmed cases and 62.7% of new cases reported in the last 24 hours, with a more significant expansion (28 of the 36 health zones affected).
- Surveillance remains suboptimal, with only 27.8% of alerts investigated, facilitating continued transmission.
- Strengthening priority interventions (surveillance, care, infection prevention and control, logistics, and community engagement) remains essential to rapidly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



EBOLA PRESIDENTIAL FORCE TASK 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Operations Manager

Dr. Kombe NGWAMA John

Editorial Team

Professor Dr. Mulangu Katulumba Félix-Médard

Dr. Katusi Biselenge Jules

Marie Afua Mayele

With technical support from the World Health Organization (WHO), Africa CDC, Bluesquaire,
Resolve Save to Live (RSTL)

Contact:

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd